

7.35.3 NMAC: proposed amendments to the practicum and training provisions

Working draft v8, July 26, 2026. Rule hearing August 28, 2026.

- This document analyzes the recommendation of Dr. Anne Metz to the Training and Education Committee dated July 17, 2026, and states it as amendment language against the proposed rule published July 23, 2026. The working redline of Denali Wilson dated July 25, 2026 is folded in, and provisions taken from it are cited to her.
- It covers 7.35.3.19, practicum requirements, and the three provisions on which the practicum depends: 7.35.3.18, educational requirements; 7.35.3.14, authorized possession; and Paragraph (5) of Subsection H of 7.35.3.20, staffing ratios. One new section is proposed; no other provision of Part 3 is addressed.
- Recommendation 1 of the Metz recommendation renames the practitioner permit to licensed provider. The renaming is carried through and marked ~~practitioner~~ **licensed provider** wherever it falls inside a provision reproduced here. Addendum C counts every other place in 7.35.3 NMAC and 7.35.2 NMAC that a conforming amendment would have to reach, so the committee can see the whole extent of it before adopting it.
- **Two decisions are put to the committee as drafted alternatives, not as questions.** Paragraph (1) of Subsection D of 7.35.3.19 is drafted in two versions. Version 1 operates under the Medical Psilocybin Act exactly as it stands today and needs no new section. Version 2 is the model the Metz recommendation describes and cannot operate until the Act is amended; proposed 7.35.3.29 states the amendment requested in its own subsection. The practicum total is 62 and 72 hours under either version.
- Where a source states a range, this draft adopts the low end and marks it, so that the committee may raise it. Where the rule as published is unclear, the published text is left exactly as it stands and the choice is set out at that provision. No figure appears anywhere in this draft that is not carried by a cited source.

The columns. Left is the rule as published, verbatim. Right is the proposed amendment: underlined green inserted, ~~struck red deleted~~, unmarked text carried forward unchanged. Metz: 6 to 8 marks a figure taken from the low end of a recommended range.

Hours Published against proposed

COMPONENT	PUBLISHED	PROPOSED	CHANGE
New Mexico module	not stated	6	+6
Psilocybin therapy module, didactic	30	68	+38
Simulated patient experience	5	5	0
Role-specific module	5	5	0
Module total, either permit	40	84	+44
Practicum, facilitator	100	62	-38
Practicum, licensed provider	120	72	-48
Supervision or consultation	10	20	+10
Program total, facilitator	150	166	+16
Program total, licensed provider	170	176	+6

The licensed provider rows take the 20 supervision hours in 7.35.3.19 C to be within the published 120. On the other reading of that subsection the published total is 190. Both readings are set out at 7.35.3.19 C.

7.35.3.14 Authorized possession, purchase, or sale of medical psilocybin

Authorized possession. Subsections A and B are amended so that a practicum participant who is not a qualified patient may lawfully be provided psilocybin under Version 2 of the first practicum stage. The permit name is conformed throughout the section. Nothing here changes who may possess or administer: those acts stay with the licensed provider, the facilitator and the healing center.

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Section heading and lead-in

7.34.3.14 AUTHORIZED POSSESSION, PURCHASE, OR SALE OF MEDICAL PSILOCYBIN BY PRACTITIONERS, FACILITATORS, HEALING CENTER OWNERS AND EMPLOYEES: Certification of a practitioner, facilitator, or healing center shall enable practitioners, facilitators, and owners and employees of healing centers to do the following, in accordance with medical psilocybin program rules:

Section heading and lead-in

~~7.34.3.14~~ **7.35.3.14** AUTHORIZED POSSESSION, PURCHASE, OR SALE OF MEDICAL PSILOCYBIN BY ~~PRACTITIONERS~~ **LICENSED PROVIDERS**, FACILITATORS, HEALING CENTER OWNERS AND EMPLOYEES: Certification of a ~~practitioner~~ **licensed provider**, facilitator, or healing center shall enable ~~practitioners~~ **licensed providers**, facilitators, and owners and employees of healing centers to do the following, in accordance with medical psilocybin program rules:

SOURCE Two changes. Correction: the heading as published reads 7.34.3.14, while the history note at the end of the same section reads 7.35.3.14. Permit name: recommendation 1 of the Metz recommendation, page 1, which renames the practitioner permit to licensed provider. Addendum C maps the full extent of that renaming.

Subsection A, Practitioners

(A) Practitioners: Practitioners may purchase and possess medical psilocybin products obtained from permitted producers, and may sell or otherwise provide medical psilocybin products to qualified patients during administration sessions conducted at a healing center location or other approved location. A practitioner may only sell or otherwise provide psilocybin products that are obtained from permitted producers.

Subsection A, Practitioners

(A) ~~Practitioners~~ **Licensed providers**: ~~Practitioners~~ **Licensed providers** may purchase and possess medical psilocybin products obtained from permitted producers, and may sell or otherwise provide medical psilocybin products to qualified patients, **or to practicum participants in accordance with 7.35.3.29 NMAC**, during administration sessions conducted at a healing center location or other approved location. A ~~practitioner~~ **licensed provider** may only sell or otherwise provide psilocybin products that are obtained from permitted producers.

SOURCE Consequential on proposed 7.35.3.29. The permit name is conformed in the same subsection.

Subsection B, Facilitators

(B) Facilitators: Facilitators may possess medical psilocybin products, for the purpose of providing those products to qualified patients in administration sessions conducted at healing centers and other approved locations.

Subsection B, Facilitators

(B) Facilitators: Facilitators may possess medical psilocybin products, for the purpose of providing those products to qualified patients, **or to practicum participants in accordance with 7.35.3.29 NMAC**, in administration sessions conducted at healing centers and other approved locations.

SOURCE Consequential on proposed 7.35.3.29.

Subsection C, Healing centers

(C) Healing centers: Owners and employees of healing centers who are registered with the department may purchase medical psilocybin products from permitted producers, may possess

Subsection C, Healing centers

Not amended.

medical psilocybin products, and may sell or otherwise administer those products to qualified patients in administration sessions conducted at the healing center or other approved locations, provided that such individuals are also designated to engage in each activity by the healing center. A healing center may only sell or otherwise provide psilocybin products that are obtained from permitted producers.

SOURCE Not amended.

Please review. This subsection conditions healing center staff authority on being “registered with the department.” No such registration is created anywhere in 7.35.3 NMAC or in 7.35.2 NMAC, so as published the condition cannot be satisfied. This draft does not fix it, because the fix is a new registration provision that belongs at 7.35.3.11 or 7.35.3.20, and neither section is within the scope of this document. The department should either create the registration or strike the condition.

7.35.3.18 Educational requirements for certifying clinicians, practitioners, and facilitators

Educational requirements. The 84-hour total is stated in one place, the New Mexico module is given an hour count, eight content areas are added to the required topic list, an incomplete sentence at Subsection F is repaired, and the permit name is conformed throughout the section.

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Section heading

7.35.3.18 EDUCATIONAL REQUIREMENTS FOR CERTIFYING CLINICIANS, PRACTITIONERS, AND FACILITATORS:

Section heading

7.35.3.18 EDUCATIONAL REQUIREMENTS FOR CERTIFYING CLINICIANS, ~~PRACTITIONERS~~ LICENSED PROVIDERS, AND FACILITATORS:

SOURCE Permit name only. Metz recommendation, page 1, recommendation 1, which renames the practitioner permit to licensed provider. Addendum C.

Subsection A, New Mexico educational module

A. Requirements for certifying clinician, practitioner, and facilitator certification: All certifying clinicians, practitioners, and facilitators who provide medical psilocybin services, including all such providers who are certified by the department on the basis of having completed an educational program from another jurisdiction, shall complete a New Mexico educational module created or approved by the department prior to applying for certification, which shall include at a minimum:

Subsection A, New Mexico educational module

A. Requirements for certifying clinician, ~~practitioner~~ licensed provider, and facilitator certification: All certifying clinicians, ~~practitioners~~ licensed providers, and facilitators who provide medical psilocybin services, including all such providers who are certified by the department on the basis of having completed an educational program from another jurisdiction, shall complete a New Mexico educational module created or approved by the department, consisting of a minimum of six didactic hours, Metz: 6 to 8 prior to applying for certification, which shall include at a minimum: *Paragraphs (1) through (5) not amended.*

SOURCE Metz recommendation, page 2, curriculum table: New Mexico module, 6 to 8 hours.

Please review. The rule sets no date by which the department must create or publish the New Mexico module, and every certification pathway in the rule is conditioned on completing it. No source in the record supplies a date, so none is drafted here. The committee should ask the department to state a date on the record.

Subsection B, certifying clinician module

Not amended.

Subsection B, certifying clinician module

Not amended.

Subsection C, Practitioner and facilitator psilocybin therapy module, lead-in

C. Requirements for initial practitioner and facilitator certification: All practitioners and facilitators shall complete a psilocybin therapy module consisting of a minimum of 30 didactic hours and 5 hours of simulated patient experience, prior to applying for certification, which shall include:

Subsection C, Practitioner and facilitator psilocybin therapy module, lead-in

C. Requirements for initial ~~practitioner~~ licensed provider and facilitator certification: All ~~practitioners~~ licensed providers and facilitators shall complete a psilocybin therapy module consisting of a minimum of ~~30~~ 68 didactic hours and 5 hours of simulated patient experience, prior to applying for certification, which shall include:

SOURCE Arithmetic. The Metz recommendation, page 1, states a total of 84 hours. 84 less Subsection A (6), less Subsection D or E (5, unchanged), less the simulated patient experience (5, unchanged) leaves 68.

Please review. The committee has to choose between two readings of the 84-hour total, and the two sources read it differently. Reading 1, drafted here, is that the 84 includes the New Mexico module (6), the role-specific module (5) and the simulated patient experience (5), which leaves 68 didactic hours for this subsection. Reading 2 is the Wilson redline, which sets this subsection at 80 didactic hours and strikes the simulated patient experience from the lead-in. Both readings produce an 84-hour program; they differ on what counts inside it. The committee should pick one, because the two cannot both be drafted.

Subsection C, required topics

- (1) Overview of practitioner/facilitator responsibilities including how to work as part of a care team;
- (2) 42 CFR part 2;
- (3) Trauma-Informed Care
- (4) Psychedelic emergencies/urgencies/medical monitoring;
- (5) Psilocybin actions, interactions and pharmacology;
- (6) Set and setting;
- (7) Ethics in therapeutic settings;
- (8) Patient-centered approaches and care;
- (9) Preparation and integration sessions;
- (10) Administration session;
- (11) Dosing;
- (12) Psychedelic de-escalation techniques;
- (13) Non-ordinary states of consciousness;
- (14) Self-care;
- (15) Research and data collection requirements;
- (16) A simulated patient experience of no less than 5 hours; and
- (17) Evaluation to demonstrate competency in the above areas.

Subsection C, required topics

- (1) Overview of ~~practitioner~~ licensed provider/facilitator responsibilities including how to work as part of a care team;
- (2) 42 CFR part 2;
- (3) Trauma-Informed Care;
- (4) Psychedelic emergencies/urgencies/medical monitoring;
- (5) Psilocybin actions, interactions and pharmacology;
- (6) Set and setting;
- (7) Ethics in therapeutic settings;
- (8) Patient-centered approaches and care;
- (9) Preparation and integration sessions;
- (10) Administration session;
- (11) Dosing;
- (12) Psychedelic de-escalation techniques;
- (13) Non-ordinary states of consciousness;
- (14) Self-care;
- (15) Research and data collection requirements;
- (16) End-of-life and palliative care;
- (17) Screening, suicidality, and crisis response;
- (18) Substance use disorder;
- (19) Post-traumatic stress disorder;
- (20) Somatic awareness and therapeutic touch;
- (21) History and traditional use of psilocybin and other psychedelics;
- (22) Individual and group treatment models;
- (23) Challenging experiences, including hallucinogen persisting perception disorder;
- ~~(16)~~ (24) A simulated patient experience of no less than 5 hours; and
- ~~(17)~~ (25) Evaluation to demonstrate competency in the above areas.

SOURCE Metz recommendation, pages 2 and 3, curriculum table, for paragraphs (16), (17), (20), (21), (22) and (23). July 17, 2026 Training and Education Committee transcript for paragraphs (18) and (19);

	both are qualifying conditions under Section 3(l) of the Medical Psilocybin Act. The Wilson redline adds the same content areas. Please review. The Metz recommendation gives an illustrative range for each content area and states that the binding minimum is the total, so no per-topic minimum is drafted here. The Wilson redline reaches the same conclusion: “I haven’t added the specific hour requirements for each because I think that is a bit much for what needs to live in regulation” (comment 13). The ranges in the recommendation are: core psychotherapy skills and ethics 25 to 30; end-of-life and palliative care 10 to 15; trauma 8; medicine, dosing and clinical research literacy 8 to 10; New Mexico module 6 to 8; treatment models 6 to 8; screening, suicidality and crisis response 5 to 8; somatic awareness and therapeutic touch 4 to 6; history and traditional use 3 to 4; challenging experiences 2 to 4. No hour figure is given anywhere for post-traumatic stress disorder or for substance use disorder. If the committee wants any content area to carry a floor of its own, it has to say which areas and what floor, because nothing in the record supplies either.
Subsection D, additional facilitator module <i>Not amended.</i>	Subsection D, additional facilitator module <i>Not amended.</i>
Subsection E, additional Practitioner module, lead-in E. Additional requirements for initial practitioner certification: All practitioners shall complete a module on psychedelic and psilocybin therapeutic approaches consisting of a minimum of five didactic hours, prior to applying for certification, which shall include:	Subsection E, additional Practitioner module, lead-in E. Additional requirements for initial practitioner licensed provider certification: All practitioners licensed providers shall complete a module on psychedelic and psilocybin therapeutic approaches consisting of a minimum of five didactic hours, prior to applying for certification, which shall include: <i>Paragraphs (1) through (3) not amended.</i> ----- SOURCE Permit name only. Metz recommendation, page 1, recommendation 1. Addendum C. The five didactic hours are not amended.
Subsection F, Practitioner and facilitator trainings, lead-in F. Practitioner and facilitator trainings: All practitioners and facilitators shall also complete and maintain proof of current certification prior to applying for medical psilocybin certification in:	Subsection F, Practitioner and facilitator trainings, lead-in F. Practitioner Licensed provider and facilitator trainings: All practitioners licensed providers and facilitators shall also complete and maintain proof of current certification prior to applying for medical psilocybin certification in: complete and maintain the following prior to applying for medical psilocybin certification: <i>Paragraphs (1) and (2) not amended. Corrects a sentence that does not complete as published. No obligation changes.</i> ----- SOURCE Correction. The sentence as published does not complete. The permit name is conformed in the same subsection.
Subsection G, continuing education G. Continuing education requirements for certifying clinicians, practitioners and facilitators: (1) Certifying clinicians: Certifying clinicians shall complete a minimum of eight hours of continuing medical education credits specific to psychedelic medicine or therapy every two years.	Subsection G, continuing education G. Continuing education requirements for certifying clinicians, practitioners licensed providers and facilitators: (1) Certifying clinicians: Certifying clinicians shall complete a minimum of eight hours of

(2) Practitioners and facilitators: Practitioners and facilitators shall complete a minimum of 20 hours of continuing education credits specific to psychedelic therapy and practice every two years. Practitioners and facilitators shall also keep current with their basic life support, or cardiopulmonary resuscitation and automated external defibrillation certification, or emergency medical technician licensure in addition to the 20 hours of continuing education credits.

continuing medical education credits specific to psychedelic medicine or therapy every two years.

(2) ~~Practitioners~~ **Licensed providers** and facilitators: ~~Practitioners~~ **Licensed providers** and facilitators shall complete a minimum of 20 hours of continuing education credits specific to psychedelic therapy and practice every two years. ~~Practitioners~~ **Licensed providers** and facilitators shall also keep current with their basic life support, or cardiopulmonary resuscitation and automated external defibrillation certification, or emergency medical technician licensure in addition to the 20 hours of continuing education credits.

SOURCE Permit name only. Metz recommendation, page 1, recommendation 1. Addendum C. The 20 continuing education hours and the life support requirement are not amended.

Subsection H, total module hours

There is no current language. This provision does not exist in the proposed rule as published.

Subsection H, total module hours

H. Total module hours: The requirements of Subsections A, C and D of this section shall together total a minimum of 84 hours for a facilitator applicant. The requirements of Subsections A, C and E of this section shall together total a minimum of 84 hours for a licensed provider applicant. The hours stated for each module in this section are minimums within that total.

SOURCE Metz recommendation, page 3: "Component ranges are illustrative; the binding minimum is the total."

7.35.3.19 Practicum requirements for practitioners and facilitators

Practicum requirements. The practicum totals, the supervision hours and the practicum sequence are amended; independent medical screening, the consultation requirement and the end-of-life checkpoint are added; the section is re-lettered A through K.

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Section heading

7.35.3.19 PRACTICUM REQUIREMENTS FOR PRACTITIONERS AND FACILITATORS:

Section heading

7.35.3.19 PRACTICUM REQUIREMENTS FOR ~~PRACTITIONERS~~ LICENSED PROVIDERS AND FACILITATORS:

SOURCE Permit name only. Metz recommendation, page 1, recommendation 1. Addendum C.

Subsection A, minimum practicum hours; administration day sessions

A. Minimum practicum hours; administration day sessions: An individual who seeks to become certified as a practitioner or facilitator shall participate in supervised practice training, otherwise referred to as a “practicum”, after completing at least half of the didactic requirements and all of the simulated patient requirements of the educational requirements. The practicum shall consist of a minimum of 100 hours of supervised practice training for facilitators and 120 hours for practitioners and shall be completed prior to applying for certification. Students shall participate in a minimum of 80 hours of administration day sessions, where students are provided the opportunity to experience, observe, or conduct supervised facilitation of administration day sessions in-person with a minimum of 14 different patients over a minimum of eight different administration and same-day sessions with the following criteria:

Subsection A, minimum practicum hours; administration day sessions

A. Minimum practicum hours; administration day sessions: An individual who seeks to become certified as a ~~practitioner~~ licensed provider or facilitator shall participate in supervised practice training, otherwise referred to as a “practicum”, after completing at least half of the didactic requirements and all of the simulated patient requirements of the educational requirements. The practicum shall consist of a minimum of ~~100~~ 62 ^{Metz: approx. 62} hours of supervised practice training for facilitators and ~~120~~ 72 ^{Metz: approx. 72} hours for ~~practitioners~~ licensed providers and shall be completed prior to applying for certification. Students shall participate in ~~a minimum of 80 hours of administration day sessions, where students are~~ administration day sessions as required by Subsection D of this section, where students are provided the opportunity to experience, observe, or conduct supervised facilitation of administration day sessions in-person with a minimum of 14 different patients over a minimum of eight different administration and same-day sessions with the following criteria: *Paragraphs (1) and (2), the patient and session minimums, not amended.*

SOURCE Metz recommendation, page 4: “approximately 62 hours for Facilitators (steps 1-3); approximately 72 hours for Licensed Providers (steps 1-4).” The permit name is conformed in the same subsection.

Please review. Three points, each of which needs a decision. First, the published phrase “half of the didactic requirements” does not identify half of what; the Wilson redline strikes the entry gate entirely, and this draft leaves it exactly as published rather than guess at the referent. Second, the published figure of 80 hours of administration day sessions cannot fit inside a 62-hour practicum, so it is struck here and the step hours in Subsection D govern instead. Third, the published minimums of 14 different patients over eight different administration and same-day sessions are carried forward unchanged in this draft, while the Wilson redline strikes them; the committee should

confirm whether those minimums are attainable within 62 hours, because if they are not, either the minimums or the total has to move.

Subsection B, minimum practicum hours; preparation and integration sessions

B. Minimum practicum hours; in-person and integration sessions: Students shall participate in a minimum of 20 hours of in-person preparation and integration sessions, where students are provided the opportunity to experience, observe, or conduct (when licensure allows) preparation or integration sessions. This shall include a minimum of six different patients during individual sessions. This shall include at least one group preparatory and one group integration session with a minimum of four or more patients.

Subsection B, minimum practicum hours; preparation and integration sessions

Not amended.

SOURCE Not amended.

Subsection C, Practitioner supervision hours

C. Practitioner supervision hours: Practitioners shall complete an additional minimum of 20 hours as a practitioner supervising facilitators during in-person administration day sessions, which shall include a minimum of two different patients during individual administration day sessions, and a minimum of one group administration day sessions with a minimum of four or more patients in the group.

Subsection C, Practitioner supervision hours

C. ~~Practitioner~~ **Licensed provider** supervision hours: ~~Practitioners~~ **Licensed providers** shall complete an additional minimum of ~~20~~ **10** Metz: 10 hours as a ~~practitioner~~ **licensed provider** supervising facilitators during in-person administration day sessions, which shall include a minimum of two different patients during individual administration day sessions, and a minimum of one group administration day sessions with a minimum of four or more patients in the group.

SOURCE Metz recommendation, page 4, step 4: "Supervisory hours, Licensed Providers only (10 hours)."

Please review. Subsection A sets 120 hours for the practitioner permit. This subsection then adds "an additional minimum of 20 hours" without saying what it is additional to, and the answer changes the published total. If the 20 hours sit inside the 120, the published program total is 170. If they sit on top of the 120, it is 190. The Metz totals of approximately 62 and 72 differ by exactly her 10 supervisory hours, which means the recommendation treats supervision as inside the practicum total, and this draft follows the recommendation for a proposed total of 176. The committee should state which reading governs so the published baseline is not ambiguous on the record.

Subsection D, practicum sequence

There is no current language. This provision does not exist in the proposed rule as published.

Subsection D, practicum sequence

D. Practicum sequence: The practicum required by Subsection A of this section consists of the following, in sequence:

(1) Initial facilitation experience. Version 1, operative under the Medical Psilocybin Act as it stands today: students shall begin the practicum by completing a minimum of two administration day sessions with qualified patients, in an observing or assisting role under the supervision of a department-permitted licensed provider. Practicum session experience shall include preparation, administration, and integration; Metz: approx. 30 hours

Version 2, which requires an amendment to the Medical Psilocybin Act: students shall begin the practicum by completing a minimum of two sessions as a facilitator in a retreat or peer-support model with well participants, in accordance with 7.35.3.29 NMAC. Practicum session experience shall include preparation, administration, and integration;

Metz: approx. 30 hours

(2) Co-facilitation experience: after completing the initial facilitation experience, students shall complete a minimum of two sessions co-facilitated with a department-permitted licensed provider, with patients presenting a low-acuity qualifying condition. Practicum session experience shall include preparation, administration, and integration;

Metz: approx. 20 hours

(3) Group practicum: direct participation in group preparation, administration, and integration sessions; and

Metz: approx. 12 hours

(4) Additional experience: students may complete the remaining balance of the hours required by Subsection A of this section in any manner of co-facilitation designed by the educational program or healing center hosting the practicum that otherwise complies with this rule. For licensed provider applicants, the supervision hours required by Subsection C of this section count toward that balance.

SOURCE Metz recommendation, pages 3 and 4, steps 1 through 4, for the sequence and the hour figures. Wilson redline for the drafting: the steps are stated as sessions, and Paragraph (4) carries the remaining balance of hours, to be completed "in any manner of co-facilitation designed by the school or healing center hosting practicums that otherwise complies with the regulations of these provisions" (Wilson redline). Paragraph (1) is drafted in two versions; see the review note.

Please review. Two matters. First, Paragraph (1) is drafted in two versions because the Medical Psilocybin Act does not authorize a person who is not a qualified patient to take psilocybin. Version 1 performs the initial experience with qualified patients and works under the Act exactly as it stands today. Version 2 performs it with well participants in a retreat or peer-support model, as the Metz recommendation describes, and cannot operate until the Act is amended. Both versions carry the same hours, so the practicum total is 62 and 72 either way. The committee should choose one and, if it chooses Version 2, adopt the statutory request stated at 7.35.3.29. Second, the Metz recommendation states each step in hours while the Wilson redline states the first two in sessions; both are carried, with session minimums drafted and the Metz hour figures shown alongside. The Wilson redline sets the practicum totals at 60 and 70, and the Metz figures of 62 and 72 are the ones drafted at Subsection A.

Subsection E, practicum location (published Subsection D)

D. Practicum location: All practicum hours shall take place in an approved healing center or other approved location.

Subsection E, practicum location (published Subsection D)

~~D. E.~~ Practicum location: All practicum hours shall take place in an approved healing center or other approved location. An approved supervisor at an approved location may host practicum students who are not enrolled at a co-located training program.

SOURCE Metz recommendation, page 4: "an approved supervisor at an approved location can host practicum students who are not enrolled at a co-located training program."

Subsection F, independent medical screening

There is no current language. This provision does not exist in the proposed rule as published.

Subsection F, independent medical screening

F. Independent medical screening required: A training program overseeing practicum treatments shall not also medically screen patients. Medical screening shall be conducted independently by the clinician with the hosting healing center or another certifying clinician not affiliated with the training program.

SOURCE Wilson redline, new subsection: "A training program overseeing practicum treatments cannot also medically screen patients."

Please review. From the Wilson redline. The stated concern is that a training program has an interest in its own students being medically cleared, because practicum hours cannot be completed until patients are cleared. Separating the two functions removes that interest. No change to the drafting is needed unless the committee disagrees with the separation.

Subsection G, practicum standards (published Subsection E)

E. Practicum standards: Practicum supervisors and students shall follow these rules as they apply to facilitation and therapy, shall comply with HIPAA and HITECH confidentiality requirements, and shall comport with applicable limits on scope of practice.

Subsection G, practicum standards (published Subsection E)

~~E.~~ **G.** Practicum standards: Practicum supervisors and students shall follow these rules as they apply to facilitation and therapy, shall comply with HIPAA and HITECH confidentiality requirements, and shall comport with applicable limits on scope of practice. *Re-lettered only. Text not amended.*

SOURCE Not amended. Re-lettered.

Subsection H, supervision and consultation; case presentation sign-off

There is no current language. This provision does not exist in the proposed rule as published.

Subsection H, supervision and consultation; case presentation sign-off

H. Supervision and consultation; case presentation sign-off:
(1) An applicant for certification shall complete a minimum of 20 hours of supervision or consultation after the practicum is completed, in addition to the practicum hours required by this section; Metz: 20 to 30
(2) sign-off is tied to case presentation. The applicant shall present a minimum of two cases of clients the applicant has personally worked with using regulated medicine. Each case presentation shall take the form of a biopsychosocial case conceptualization, including a discussion of presenting concerns, risk factors, supportive factors, treatment considerations, and recommendations for aftercare; and
(3) the consultant or supervisor shall complete a standardized evaluation form on each case presented.

SOURCE Metz recommendation, pages 4 and 5: 20 to 30 hours of supervision or consultation, with sign-off tied to two presented cases and a standardized evaluation form.

Please review. The two sources put this requirement in different places and state it in different units. The Metz recommendation states 20 to 30 hours of supervision or consultation, drafted here at the low end of 20, with sign-off tied to two presented cases.

	The Wilson redline instead replaces the 10 mentoring hours at 7.35.3.17 A with two student case consultations, states the requirement in cases rather than hours, and strikes the 12-month validity period. Dr. Metz, Ms. Wilson and Dr. Leeman should settle two things: whether the requirement is stated in hours, in cases, or in both, and whether it sits here at 7.35.3.19 H or at 7.35.3.17 A. Note that 7.35.3.17 A is outside the scope of this document, so if they choose 7.35.3.17 A the amendment has to be drafted there.
Subsection I, end-of-life practice <i>There is no current language. This provision does not exist in the proposed rule as published.</i>	Subsection I, end-of-life practice <u>I. End-of-life practice: Before serving end-of-life participants independently, a licensed provider or facilitator shall complete at least one co-facilitated end-of-life case and present at least one end-of-life case in supervision or consultation.</u> SOURCE Metz recommendation, page 5, end-of-life checkpoint.
Subsection J, waiver of practicum requirements (published Subsection F) F. Waiver of practicum requirements: The department may otherwise waive, temporarily suspend, or reduce the practicum requirements for individuals applying for certification, in order to facilitate the certification of individuals trained by other government-approved programs, and to build the initial infrastructure of the program.	Subsection J, waiver of practicum requirements (published Subsection F) F. <u>J.</u> Waiver of practicum requirements: The department may otherwise waive, temporarily suspend, or reduce the practicum requirements for individuals applying for certification, in order to facilitate the certification of individuals trained by other government-approved programs, and to build the initial infrastructure of the program. <i>Re-lettered only. Text not amended.</i> SOURCE Not amended. Re-lettered.
Subsection K, waiver for applications received by December 31, 2027 (published Subsection G) G. Waiver of practicum hours requirement for applications received by December 31, 2027: An applicant for certification shall not be required to satisfy the full New Mexico practicum hours requirement if the applicant: (1) Applies for certification by December 31, 2027; (2) Completes the didactic requirements by December 31, 2027; (3) Graduates from an educational program that the department certifies by December 31, 2027 or that the department has included on the department-approved list of educational programs by December 31, 2027; and (4) Demonstrates completion of at least 40 hours of contact time through logs or other records of the sessions, including: (a) A minimum of two separate individual sessions including the appointments for preparation, administration and integration; and (b) A minimum of one group session including the appointments for preparation, administration, and integration.	Subsection K, waiver for applications received by December 31, 2027 (published Subsection G) G. <u>K.</u> Waiver of practicum hours requirement for applications received by December 31, 2027: <i>Re-lettered only. Paragraphs (1) through (4) not amended.</i> SOURCE Not amended. Re-lettered. Please review. Two provisions set different group session minimums for the same 40 hours of contact time. Paragraph (4)(b) of this subsection requires “A minimum of one group session” while Subsection D(1) of 7.35.3.10 requires “two separate group sessions”. Neither is amended here, because 7.35.3.10 is outside the scope of this document and the two have to be reconciled together. The department should align them.

7.35.3.20 Requirements for healing centers and other approved locations

Requirements for healing centers and other approved locations. The section heading number is corrected and the permit name is conformed in Paragraph (5) of Subsection H. No staffing ratio is changed, and the qualified student provision is carried forward exactly as the department wrote it.

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Section heading and lead-in

7.34.3.20 REQUIREMENTS FOR HEALING CENTERS AND OTHER APPROVED LOCATIONS: A healing center and a registrant of another approved location shall comply with the following requirements:

Section heading and lead-in

~~7.34.3.20~~ **7.35.3.20** REQUIREMENTS FOR HEALING CENTERS AND OTHER APPROVED LOCATIONS: A healing center and a registrant of another approved location shall comply with the following requirements: *Subsections A through G and I not amended. Of Subsection H, only Paragraph (5) is reproduced.*

SOURCE Correction. The heading as published reads 7.34.3.20, while the history note at the end of the same section reads 7.35.3.20.

Subsection H(5), staffing ratios

(5) Includes procedures to ensure that there are practitioners and facilitators present at all times during an administration session with a minimum of one practitioner and one facilitator for individual patient sessions; and in group administration sessions a minimum of one practitioner for every eight patients and a minimum of one facilitator or qualified student for every two patients. For purposes of the foregoing provision, a student shall be deemed qualified if they are registered with a certified educational program and they have completed at least 50 hours of their practicum. Exception: the department may waive or decrease this ratio if the department determines that the ratio specified presents a barrier for patients and that safety concerns are otherwise alleviated.

Subsection H(5), staffing ratios

(5) Includes procedures to ensure that there are ~~practitioners~~ **licensed providers** and facilitators present at all times during an administration session with a minimum of one ~~practitioner~~ **licensed provider** and one facilitator for individual patient sessions; and in group administration sessions a minimum of one ~~practitioner~~ **licensed provider** for every eight patients and a minimum of one facilitator or qualified student for every two patients. For purposes of the foregoing provision, a student shall be deemed qualified if they are registered with a certified educational program and they have completed at least 50 hours of their practicum. Exception: the department may waive or decrease this ratio if the department determines that the ratio specified presents a barrier for patients and that safety concerns are otherwise alleviated.

SOURCE Permit name only. Metz recommendation, page 1, recommendation 1. Addendum C. No ratio is amended, and the qualified student provision is carried forward exactly as the department published it.

Please review. No training permit is proposed anywhere in this draft, and none is needed. At the July 17, 2026 Training and Education Committee meeting the department stated that a training permit “really wouldn’t be necessary with the model that we have” because a student is already covered “as them being a registered student with an educational program where they have that ability to be a part of those training sessions, including to conduct them under supervision” and that “those students already will have that kind of protection ability to be there as a part of being registered as a student with an educational program.” The department undertook to count students toward the ratio “rather than needing to create an entire other permit or certification level.” This paragraph as published is the result. Nothing further is required: a student conducts facilitation under supervision, while the acts 7.35.3.14 authorizes, purchase, possession, sale and administration, remain with the licensed provider, the facilitator and the healing

center. The rule is consistent with that division throughout. 7.35.3.19 B permits a student to “experience, observe, or conduct (when licensure allows) preparation or integration sessions.” 7.35.3.20 D permits “students completing their practicums” to be present at an administration session. 7.35.3.17 C(2)(e) requires a program to record the “Names of facilitators supervised by practitioner students during practicum.” The phrase “training permit” appears nowhere in the rule as published, and the qualification parameter is the qualified student definition in this paragraph.

7.35.3.29 Proposed new section, in two versions. Practicum participants who are not qualified patients

The first stage of the practicum, in two versions. Version 1 needs no new section and works under the Medical Psilocybin Act as it stands today. Version 2 is the model the Metz recommendation describes, and Subsection D of it states the statutory amendment that would be required.

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Version 1, no new section required

There is no current language. This provision does not exist in the proposed rule as published.

Version 1, no new section required

No new section. Under Version 1 of Paragraph (1) of Subsection D of 7.35.3.19 NMAC the entire practicum is performed with qualified patients, which the Medical Psilocybin Act already authorizes, so nothing needs to be added to Part 3. If the committee adopts Version 1, this section is deleted and the practicum is lawful on the day the rule takes effect.

SOURCE Medical Psilocybin Act, Section 5(B)(2), which exempts “a clinician administering or a qualified patient taking psilocybin in an approved setting”. Version 1 keeps the practicum inside that exemption, so no new section is required.

Please review. This is the version that operates under the Medical Psilocybin Act exactly as it stands today, and it requires no new section at all. Section 5(B)(2) of the Act exempts “a clinician administering or a qualified patient taking psilocybin in an approved setting”, and Section 3(H) defines a qualified patient by diagnosis with a qualifying condition, so psilocybin may lawfully be taken only by a qualified patient. Version 1 therefore performs the initial practicum experience with qualified patients, in the same settings and under the same supervision as the rest of the practicum. The cost is that a student first encounters a supervised administration session with a clinical population rather than before one, which is the sequencing the Metz recommendation was designed to avoid. The benefit is that the practicum is lawful on the day the rule takes effect. If the committee adopts Version 1, delete proposed 7.35.3.29 entirely and adopt Paragraph (1) of Subsection D of 7.35.3.19 in its Version 1 form.

Version 2, new section conditioned on a statutory amendment

There is no current language. This provision does not exist in the proposed rule as published.

Version 2, new section conditioned on a statutory amendment

7.35.3.29 PRACTICUM PARTICIPANTS WHO ARE NOT QUALIFIED PATIENTS:
A. Purpose: This section provides for the stage of the practicum described in Version 2 of Paragraph (1) of Subsection D of 7.35.3.19 NMAC to be conducted with practicum participants who are not qualified patients, so that a student gains supervised exposure to non-ordinary states before working with clinical populations.
B. Practicum participant: A practicum participant is an individual who is not a qualified patient and who takes part in a practicum session in a retreat or peer-support model. Preparation and integration sessions are required for each participant.
C. Effect: This section does not operate unless and until the Medical Psilocybin Act authorizes a person who is not a qualified patient to take medical psilocybin in an approved setting under department rule. Until that authorization exists, no session may be conducted

under this section.

D. Statutory amendment requested: This section is drafted on the express request that Section 5 of the Medical Psilocybin Act be amended. Paragraph (2) of Subsection B of that section exempts from criminal and civil liability a clinician administering or a qualified patient taking psilocybin in an approved setting. The amendment requested is to extend that exemption to a practicum participant taking medical psilocybin in an approved setting under department rule, and to a certificant or student administering or providing medical psilocybin to such a participant. No lesser change reaches this stage of the practicum, because Subsection D of Section 5 of the Act protects presence only.

SOURCE Metz recommendation, page 3, step 1: "Initial facilitation experience with well participants (approximately 30 hours). Two to three sessions as a facilitator in a retreat or peer-support model." Version 2 requires an amendment to the Medical Psilocybin Act, stated in Subsection D of the section.

Please review. This is the version the Metz recommendation describes, and it cannot operate until the Medical Psilocybin Act is amended. This draft states that requirement expressly rather than burying it. Section 5(B)(2) of the Act exempts only "a clinician administering or a qualified patient taking psilocybin in an approved setting", and a practicum participant who is not a qualified patient falls outside it. Subsection D of the proposed section names the amendment required: the exemption in Section 5 has to reach a practicum participant taking psilocybin in an approved setting under department rule. Subsection C conditions the whole section on that amendment, so if the section is adopted and the Act is never amended, the section simply does not operate and nothing unlawful is authorized. The committee should decide whether to pursue the statutory amendment. If it does not, adopt Version 1.

Addendum A Provisions on which the practicum operates

Provisions on which 7.35.3.19 operates, and whether this draft amends each.

Provision	Page	What the practicum needs from it	In this draft
7.35.2.7 NMAC	n/a	Supplies every defined term used in Part 3, by force of 7.35.3.7	Not amended here. Renaming the permit would require amending the definition at P(7); 7.35.2 NMAC is a separate rulemaking. Facilitator, healing center, certifying clinician and student are used in Part 3 and defined in neither part. Addenda B and C
Medical Psilocybin Act, Section 5	n/a	The criminal and civil exemption. Names producers, clinicians and qualified patients, and protects presence only for anyone else	Statute. Not reachable by rule
7.35.3.9 C(3), E, F	3	Certification applications require documentation of the completed practicum, and E(1) sets the practitioner licensure predicate	Not amended
7.35.3.10 D	5	Out-of-jurisdiction 40-hour practicum waiver	Not amended. Disagrees with 7.35.3.19 K
7.35.3.11 A, B	5 to 7	Healing centers and other approved locations. The practicum may only happen in them	Not amended
7.35.3.12 A	7	Educational program certification	Not amended
7.35.3.13 B	8	Facilitator scope of work, which limits what the practicum can usefully train	Not amended
7.35.3.14	9	Authority to possess and administer	AMENDED. Heading, Subsections A and B
7.35.3.15 A(4)	9	Practicum site agreements need prior written approval, with no deadline on the department	Not amended
7.35.3.17 A	10 to 11	The 10 mentoring hours the consultation requirement would replace	Not amended
7.35.3.17 B, C	11	Test-out, graduation conditions, and the practicum records a program must keep	Not amended
7.35.3.18	11 to 12	The didactic requirement the practicum entry gate is measured against	AMENDED. Heading and Subsections A, C, E, F and G
7.35.3.19	12 to 13	The practicum	AMENDED. Re-lettered A to K
7.35.3.20 D	14	Students may be present at an administration session	Not amended
7.35.3.20 H(5)	14	Staffing ratio, which counts a qualified student	AMENDED. Permit name only. No ratio changed
7.35.3.22	15	Prohibitions, reaching "the qualified patient or certificant"	Not amended

7.35.3.27	16 to 19	Discipline and appeal	Not amended
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Proposed new provisions

Provision	What it does	Source	If dropped
7.35.3.18 H	States the 84-hour total in one place	Metz recommendation, page 3	Module hours survive individually; no total is stated anywhere
7.35.3.19 D	The four-step practicum sequence	Metz recommendation, pages 3 to 4; Wilson redline for the drafting	Practicum totals survive; the sequence is lost
7.35.3.19 F	Separates medical screening from the training program overseeing the practicum	Wilson redline	A program may clear its own students for the treatments that generate their hours
7.35.3.19 H	20 consultation hours, two presented cases, standardized evaluation form	Metz recommendation, pages 4 to 5	7.35.3.17 A survives at 10 hours with no requirement to have seen a client
7.35.3.19 I	End-of-life checkpoint	Metz recommendation, page 5	No end-of-life competency gate remains
7.35.3.29 Version 2 only	Lets the first practicum stage run with participants who are not qualified patients, and states the statutory amendment that would be required	Metz recommendation, step 1, page 3; Medical Psilocybin Act, Section 5	Version 1 governs, the first stage runs with qualified patients, and no statutory work is needed

No training permit is proposed. At the July 17, 2026 Training and Education Committee meeting the department stated that a training permit "really wouldn't be necessary with the model that we have" and undertook to count students toward the staffing ratio instead. Paragraph (5) of Subsection H of 7.35.3.20 as published does that. See the note at that paragraph.

The one statutory request in this document

Everything else in this draft can be adopted by rule. One thing cannot. If the committee wants the first practicum stage to run with well participants, as the Metz recommendation describes, the Medical Psilocybin Act has to be amended, and this document asks for that amendment in plain terms rather than leaving it implied.

What the Act says now	Why the first stage falls outside it	The amendment requested	If it is not pursued
Paragraph (2) of Subsection B of Section 5 makes lawful the conduct of "a clinician administering or a qualified patient taking psilocybin in an approved setting". Subsection D of the same section protects a person "for simply being in the presence of the medical use of psilocybin"	A well participant is not a qualified patient, because Subsection H of Section 3 defines a qualified patient by diagnosis with a qualifying condition. Presence is protected; taking psilocybin is not	Extend the exemption in Section 5 to a practicum participant taking medical psilocybin in an approved setting under department rule, and to a certificant or student administering or providing it to such a participant	Adopt Version 1. The first stage runs with qualified patients, the practicum total stays at 62 and 72 hours, and proposed 7.35.3.29 is deleted

Addendum B Terms used in Part 3 and defined nowhere

7.35.3.7 provides in full: "The definitions in 7.35.2.7 NMAC apply to this part." 7.35.2 NMAC was adopted effective June 23, 2026. The terms below are used in Part 3. Apart from the permit title, no amendment to the definitions is proposed in this document.

Term used in Part 3	In 7.35.2.7?	What 7.35.2.7 has	Consequence
Facilitator	No. Zero occurrences in 7.35.2 NMAC	"Guide" an individual who has completed training and education approved by the department to be able to assist practitioners during the administration sessions and who has been registered with the department	Every facilitator provision in Part 3 rests on an undefined term. A guide holds no professional license, so a facilitator is not a "clinician" under Section 3(B) of the Act, while 7.35.3.14 B authorizes facilitators to possess and provide
Healing center	No	"Approved location" means a location approved by the department for psilocybin administration sessions	Healing centers are certified and regulated throughout Part 3 with no definition
Certifying clinician	No	"Clinician" means an approved health care provider licensed in New Mexico who holds a certification from the department to provide medical services to qualified patients	Probably the same role renamed
Student	No	Nothing	Carries consequence at 7.35.3.19, 7.35.3.20 D and 7.35.3.20 H(5)
Practitioner	Yes	"Practitioner" means an individual who is a licensed healthcare professional who is certified by the department to provide medical psilocybin integrative therapy, supervise guides, and who has completed department required trainings	Carries the same licensure predicate as the Act's "clinician". Renamed by this draft. Addendum C

Addendum C The permit name, and every place the renaming reaches

Recommendation 1 of the Metz recommendation, page 1, renames the practitioner permit to licensed provider. This draft carries the renaming. It is marked ~~practitioner~~ **licensed provider** wherever it falls inside a provision reproduced in this document. The term is defined at Paragraph (7) of Subsection P of 7.35.2.7 NMAC, and 7.35.3.7 NMAC provides in full: "The definitions in 7.35.2.7 NMAC apply to this part." Renaming the permit is therefore an amendment to 7.35.2 NMAC plus a conforming pass over 7.35.3 NMAC. The tables below count every occurrence in both parts, so that the committee can see the whole surface.

Two drafting notes. First, the Metz recommendation writes "Licensed Provider" in title case; this draft writes "licensed provider" in lower case, because the rule as published writes "practitioner" in lower case except in headings and at the start of a subsection. Second, the renaming reaches no hour count and no practicum requirement. Those attach to the permit type, and what the permit is turns on the licensure predicate at Paragraph (1) of Subsection E of 7.35.3.9 NMAC, which already requires a New Mexico professional license. Adopting or declining the renaming changes nothing else in this document.

7.35.3 NMAC, section	Occurrences	Reached by this draft	Note
Part title	1	No	"PART 3 PATIENTS, CERTIFYING CLINICIANS, PRACTITIONERS, FACILITATORS, HEALING CENTERS, OTHER APPROVED LOCATIONS, AND EDUCATIONAL PROGRAMS"
7.35.3.2 Scope	1	No	Who the part applies to
7.35.3.6 Objective	1	No	
7.35.3.9 Certification requirements	5	No	Includes the section heading and the licensure predicate at E(1)
7.35.3.10 Application process	5	No	Includes the section heading and the out-of-jurisdiction pathway
7.35.3.11 Healing centers	3	No	
7.35.3.12 Educational programs	1	No	
7.35.3.13 Requirements and prohibitions	12	No	The largest block outside this draft. Includes the section heading and the scope of work
7.35.3.14 Authorized possession	6	Yes, all 6	Heading and Subsection A
7.35.3.17 Educational programs	2	No	Includes the mentoring provision at Subsection A
7.35.3.18 Educational requirements	14	Yes, all 14	Heading and Subsections A, C, E, F and G
7.35.3.19 Practicum requirements	6	Yes, all 6	Heading and Subsections A and C
7.35.3.20 Healing centers	3	Yes, all 3	All three are in Paragraph (5) of Subsection H

7.35.3.26 Voluntary withdrawal	1	No	
7.35.3.27 Disciplinary actions	5	No	Includes the section heading
Total, 7.35.3 NMAC	66	29 reached, 37 not	Fourteen of the twenty-eight sections carry the term

7.35.2 NMAC, section	Occurrences	Reached by this draft	Note
7.35.2.7 Definitions	3	No	The definition itself at P(7); also inside the definitions of "certification" at C(3) and "guide" at G(3)
7.35.2.10 General producer requirements	1	No	
7.35.2.14 Packaging and labeling	3	No	The product information document duties
7.35.2.15 General tracking requirements	1	No	
7.35.2.19 Required testing	2	No	Includes the failed-lot notification duty
7.35.2.24 Transportation	1	No	
7.35.2.26 Disciplinary actions	1	No	
Total, 7.35.2 NMAC	12	0 reached	7.35.2 NMAC was adopted effective June 23, 2026 and is not before this hearing

Please review. The renaming is all or nothing. If it is adopted, the conforming amendment has to reach all 66 occurrences in 7.35.3 NMAC and all 12 in 7.35.2 NMAC, including the definition at Paragraph (7) of Subsection P of 7.35.2.7 NMAC. This document reaches 29. If the committee adopts the renaming, the remaining 49 are a conforming pass, and 7.35.2 NMAC is a separate rulemaking. If the committee declines it, strike every ~~practitioner~~ licensed provider mark in this document; nothing else in the document depends on it.

Addendum D Sections not addressed

Part 3 has twenty-eight sections. This draft addresses the practicum and the provisions on which it operates. The sections below are not addressed. Findings are recorded in the concerns inventory of July 23, 2026, which lists five blocking and twenty-three material findings across the part.

Group	Sections	Recorded findings
Terminology	7.35.3.7, and 7.35.2.7 NMAC	Facilitator, healing center, certifying clinician and student undefined. Addendum B
Educational programs	7.35.3.12, .15, .16, .17	The evaluator conflict rule disqualifies the evaluators a program must hire; the third-party evaluation deferral expires the day it becomes available; no deadline binds the department on instructor changes; the denial grounds were deleted; the evaluator qualification standard may be unmeetable; mentoring is unfunded and untied to certification; the test-out price cap assumes per-module pricing
Out-of-jurisdiction pathway	7.35.3.10	The two 40-hour waivers disagree; the 2027 waiver may cancel itself; the approved list is empty at launch and populates only retroactively; individuals must produce an evaluation only a program can commission
Locations and oversight	7.35.3.11, .20, .21	Healing center staff authority hangs on a registration that does not exist; outdoor locations need no AED while healing centers do; the patient roster and the patient-interview authority; other approved locations have a 90-day term and no renewal path
Patients, applicants, process	7.35.3.8, .9, .13, .22 to .27	Facilitator scope conflicts with the practicum and the ratios; complainants lose confidentiality; the benefit-risk attestation; renewals can lapse while pending; the re-application clause reads as a deadline; two review tracks with inconsistent scope; a suspended certificant can be out of practice more than 135 days
Mechanical	Throughout	Five section headings read 7.34.3 instead of 7.35.3, two of which are corrected here; two sections carry a real effective date while the rest carry placeholders; a numbering gap at 7.35.3.9 D

Sources. Rule as published: *rules-draft-2026-07-23-published.pdf*, 19 pages. Recommendations: Dr. Anne Metz, *Recommendations on Education and Training Requirements for Facilitators and Licensed Providers*, July 17, 2026, with the one-page summary and the committee slide deck of the same date. Working redline: Denali Wilson, *NMAC 7.35.3*, July 25, 2026, tracked changes and comments as exported from the document file. July 17, 2026 transcripts, morning Advisory Board and afternoon Training and Education Committee, both labeled "UNOFFICIAL AUTO-GENERATED TRANSCRIPT. NO SPEAKER ATTRIBUTION." Statute: the Medical Psilocybin Act, cited throughout by its own section numbers. The text consulted is the Act as passed in the 2025 regular session; its codification in NMSA 1978 has not been verified and no NMSA section number is asserted anywhere in this document. 7.35.2 NMAC as adopted effective June 23, 2026.

Method. The left column reproduces the rule as published, verbatim, with line breaks introduced by the PDF collapsed to single spaces and no other alteration. Each block was checked against the text layer of the published rule.

Status. Working draft v8. Not a filing, not submitted, and not adopted rule text.